

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Desmond R. Achterberg DOB: 1966-03-08 (age 60) Member ID: MBR-90031

Sex: M

Requesting Provider

Practice: Harborview Electrophysiology Partners

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-10-22

Level of care requested: INPATIENT

Made-up data for a software prototype. Not a real patient.

PCP referral letter

TIDELINE FAMILY MEDICINE

08/20/2026

To: Harborview Electrophysiology Partners

Re: Desmond Achterberg, DOB 03/08/1966

Dear colleagues,

I am referring Mr. Achterberg for ICD consideration. He has a cardiomyopathy with EF in the low 30s and a very strong family history of premature coronary disease - his father had an MI at 49 and later died of ischemic cardiomyopathy, and a paternal uncle had CABG at 55. I suspect ischemic cardiomyopathy. He has been wearing a LifeVest since the spring.

He also has HTN, OSA on CPAP, and obesity (BMI 34).

Thank you for seeing him.

Corinne Vasquez-Albee, DO

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EP consultation

HARBORVIEW ELECTROPHYSIOLOGY PARTNERS

Consultation 09/17/2026

Pt: Achterberg, Desmond R. 60M

HPI: On 03/08/2026 pt collapsed while on a treadmill at his gym. Bystander CPR, AED analyzed VF and delivered 1 shock with ROSC. Emergent cath that day showed nonobstructive CAD, no culprit, no intervention. Troponin peak attributed to arrest/CPR, no MI. CMR 03/11: LVEF 31%, patchy mid-wall LGE, no myocarditis by T2. No reversible cause found (K, Mg normal, no drugs, no QT prolongation). Neurologically intact on discharge, no anoxic injury. Discharged 03/14 with wearable defibrillator and GDMT. No WCD shocks since.

Now: mild DOE on hills and stairs. NYHA class II. No syncope.

FHx: father MI at 49 and died of ischemic CM; paternal uncle CABG. No family SCD.

Data: TTE 09/12/26 LVEF 33%.

Assessment: Resuscitated VF arrest without reversible cause, with persistent LV dysfunction. Coronaries nonobstructive - not ischemic CM despite family history.

Indication: SECONDARY prevention ICD (survivor of VF cardiac arrest).

Plan: transvenous single-chamber ICD; SDM visit; admit.

Signed: Ingrid Halloran-Obi, MD

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Cardiac catheterization report

HARBORVIEW MEDICAL CENTER - EMERGENT CARDIAC CATH

Date: 03/08/2026 Pt: ACHTERBERG, DESMOND (post-arrest, intubated)

LM: normal. LAD: 30% mid. LCx: 20% OM1. RCA: minor irregularities.

No thrombus, no culprit. LVEDP 22.

Conclusion: Nonobstructive CAD. No PCI performed.

Operator: K. Sandoval, MD

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Echocardiogram report

HARBORVIEW ECHO LAB 09/12/2026

ACHTERBERG, DESMOND R

LVEF (3D): 33%

LVIDd 6.0. Global hypokinesis, no regional WMA.

RV normal. No significant valve disease.

Compared to CMR 03/2026 (31%): no meaningful change.

Read by: A. Fontaine, MD

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Shared decision making visit

Harborview EP - Shared Decision Making Encounter

09/24/2026 Desmond Achterberg (wife present)

Clinician: Lars Nygaard, PA-C

Tool: evidence-based ICD decision aid (video + booklet).

We reviewed recurrence risk after VF arrest, how the ICD treats VF, risks of implant, inappropriate shocks, driving rules, and that the LifeVest is not a long-term option.

Patient chose ICD; wife agrees.

L. Nygaard PA-C

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Medication list

Achterberg, Desmond - Medication list 09/17/2026
metoprolol succinate 100 mg daily start 03/12/2026
sacubitril/valsartan 49/51 BID start 03/12/2026
spironolactone 25 mg daily start 03/12/2026
empagliflozin 10 mg daily start 04/2026
atorvastatin 40 mg daily start 2021
amlodipine stopped 03/2026

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Pre-admission plan

HARBORVIEW EP - PRE-ADMISSION PLAN 10/01/2026

Patient: Desmond R. Achterberg MBR-90031

Procedure: ICD (CPT 33249) Admit: 10/22/2026 inpatient

Expected stay: 2 midnights.

Supporting factors: severe OSA (AHI 46) on CPAP - post-sedation monitoring; obesity BMI 34 w/ difficult airway history; HTN; transition off WCD requires telemetry until device check POD1.

Limiting conditions: none. Neuro intact, no terminal illness, sinus rhythm (no AF), clinically stable.

No MI, PCI or CABG.

I. Halloran-Obi MD

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